



Diabetes: Australian facts

Web report | Last updated: **12 Dec 2024** | Topic: [Diabetes](#) | [Media release](#) | **Citation**

Latest edition

All diabetes

Page highlights:

How many people are living with diabetes in Australia?

- More than 1.3 million (1 in 20) Australians were living with diabetes in 2021.
- Diabetes was more common among older Australians with almost 1 in 5 aged 80–84 living with diabetes.
- Males were 1.3 times as likely to be living with diabetes compared with females.
- 7.9% of Aboriginal and Torres Strait Islander (First Nations) people were living with diabetes in 2018–19.

How many people are newly diagnosed with diabetes in Australia?

- 49,900 people were newly diagnosed (incidence) with diabetes in 2021.
- The incidence of diabetes increased with increasing age, peaking in the 65–69 age group for both males and females.
- Since 2000, there has been a 43% decline in the age-standardised incidence rate for diabetes.
- There were around 2,200 new cases of diabetes among First Nations people in 2021.

How many people are living with diabetes in Australia?

More than 1.3 million (1 in 20) Australians were living with diabetes and currently registered with the linked National Diabetes Services Scheme (NDSS) and Australasian Paediatric Endocrine Group (APEG) state-based registers in 2021. This includes people with type 1 diabetes, type 2 diabetes and other diabetes, but excludes gestational diabetes.

Variation by age and sex

In 2021, the prevalence of diabetes, according to linked NDSS and APEG data, increased with increasing age:

- Almost 1 in 5 (19.5%) Australians aged 80–84 (22.1% males and 17.2% of females) (Figure 1) were living with diabetes in 2021, which was almost 30 times as high as those aged under 40 (0.7%).
- Diabetes was 1.3 times as common among males as females, after controlling for age.

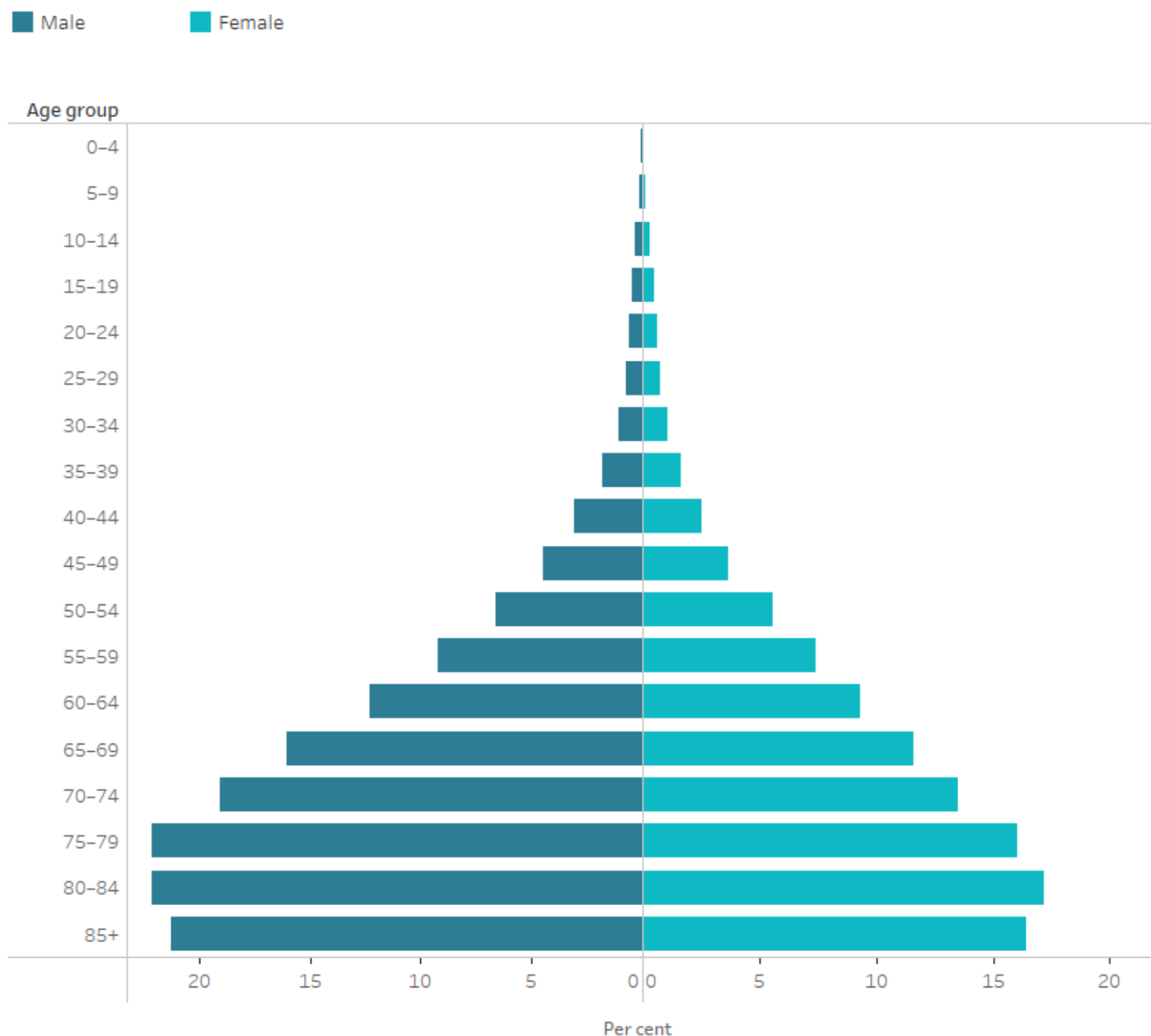
Figure 1: Prevalence of diabetes, by age and sex, 2021

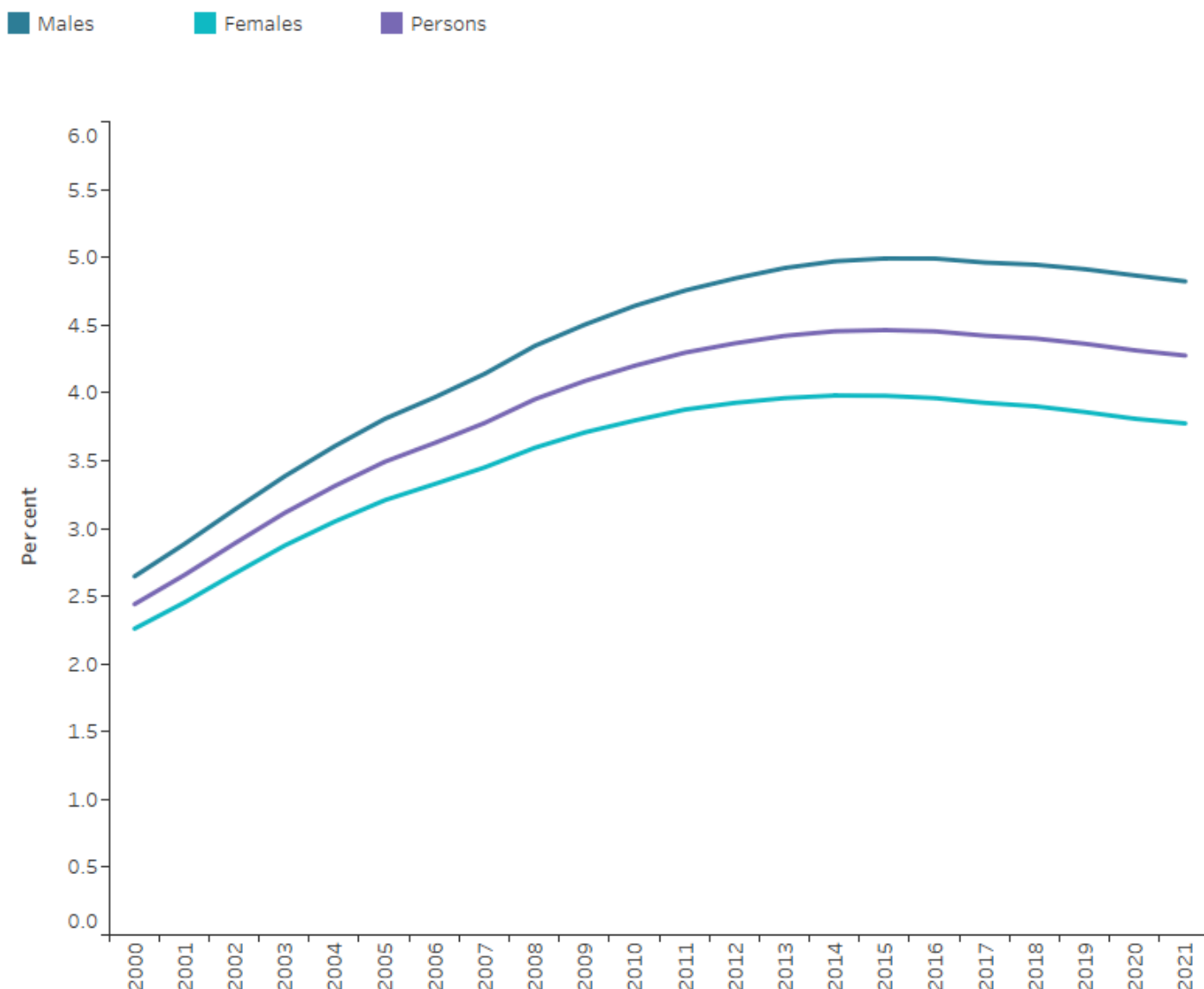
Chart: AIHW. Source: AIHW analysis of linked National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

<https://www.aihw.gov.au>

Trends over time

The number of people living with diabetes in Australia increased almost 2.8-fold between 2000 and 2021, from 460,000 to 1.3 million.

Based on linked NDSS and APEG data, the age-standardised prevalence rate increased by almost 80% between 2000 and 2021, however, rates have remained relatively stable since 2011 for both males and females (Figure 2).

Figure 2: Prevalence of diabetes, by sex, 2000–2021

Note: Age-standardised to the 2001 Australian Standard Population.

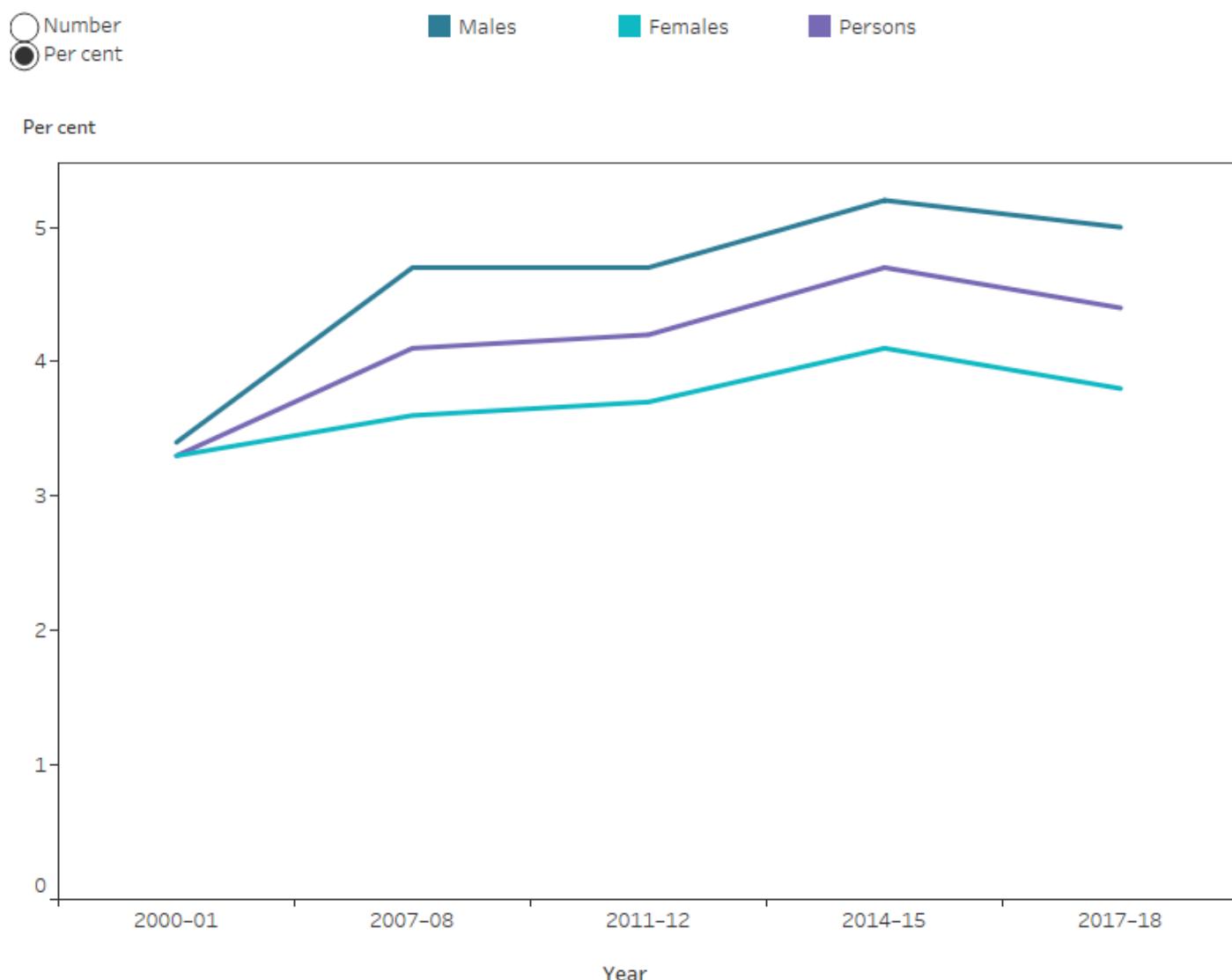
Chart: AIHW. Source: AIHW analysis of linked National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

<https://www.aihw.gov.au>

Similar patterns were found in the ABS NHS. The age-standardised prevalence rate of diabetes increased by 33% between 2001 and 2017–18. The rate of self-reported diabetes remained stable between 2014–15 and 2017–18 (Figure 3).

Data from the most recent ABS NHS 2022 show a slight increase in crude prevalence from 4.9% in 2017–18 to 5.3% in 2022 with males and females having similar rates of diabetes (5.8% and 4.9%, respectively) (ABS 2022).

Figure 3: Prevalence of self-reported diabetes from the National Health Survey, by sex, 2001 to 2017-18



Notes

1. Respondent ever told by a doctor or a nurse that they had diabetes. Includes diabetes type 1, type 2 and type unknown. Data for 2004-05 are not available using this definition.

2. The estimates also include persons who reported they had diabetes but that it was not current at the time of interview.

3. Per cent age-standardised to the 2001 Australian population.

Chart: AIHW. Sources: AIHW analysis of ABS 2003; ABS 2009; ABS 2013b; ABS 2016; ABS 2019.

<https://www.aihw.gov.au/>

Limitations of estimating diabetes prevalence

Data presented from the linked National Diabetes Services Scheme (NDSS) and Australasian Paediatric Endocrine Group (APEG) data (Figure 2) and Australian Bureau of Statistics (ABS) National Health Survey (NHS) (Figure 3) are likely to underestimate the true prevalence of diabetes in the Australian population. This is because:

- Both data sources are based on people who have received a formal medical diagnosis of diabetes. However, Australian studies have shown that many people are living with undiagnosed type 2 diabetes. For example, in the 1999–2000 AusDiab Study, half of all diabetes were undiagnosed (Dunstan et al. 2001). In the 2011–12 Australian Bureau of Statistics (ABS) Australian Health Survey (AHS), which collected blood glucose data, 20% of participating adults aged 18 and over had undiagnosed type 2 diabetes prior to the survey (ABS 2013a).

- Registration with the NDSS is voluntary and eligible people with type 2 diabetes are more likely to register if they access subsidised diabetes consumables to monitor their diabetes at home or require insulin. Some people may be diagnosed with diabetes and choose not to register with the scheme.
- First Nations people are under-represented on the NDSS (see [Using the NDSS for reporting on First Nations people](#)).

Despite these limitations, these data sources provide the best picture into the number of people living with diabetes in Australia to monitor changes in populations at risk, and trends over time. Further research is required to examine whether the proportion of people with undiagnosed type 2 diabetes in Australia has changed over time and the impact of this on the prevalence of disease in Australia.

Variation between population groups

First Nations people

Around 7.9% of Aboriginal and Torres Strait Islander (First Nations) people (64,100) were living with diabetes according to self-reported data from the ABS 2018–19 National Aboriginal and Torres Strait Islander Health Survey (NATSIHS) (ABS 2019b). This is similar to the 7.7% reported in the equivalent survey in 2012–13 (ABS 2014).

After controlling for differences in the age structure of the populations, based on self-reported and measured results, First Nations people were almost 3 times as likely to be living with diabetes as non-Indigenous Australians.

Using the NDSS for reporting on First Nations people

The representation and accuracy of data relating to Aboriginal and Torres Strait Islander (First Nations) people on the NDSS is influenced by a range of factors, which may reduce the accuracy of reporting on these communities and prevent the use of the NDSS for reporting on prevalence by Indigenous status.

- Before 2005, data entry for the NDSS registration form for Indigenous status coded all 'unknown' or 'not stated' responses to the Indigenous status question as 'non-Indigenous'. In 2005, the NDSS database was amended to add an extra value to the Indigenous status variable to indicate 'inadequate/not stated' where Indigenous status was not known. As a result of this issue, Indigenous status cannot be determined for 98% of people registered on the NDSS prior to 2005.
- First Nations people may not register with the NDSS at all. Where First Nations people live in rural, remote and very remote locations, NDSS Access Points may be limited, or consumables may be obtained through bulk supplies provided to health services through the NDSS. Diabetes-related products can also be accessed through other programs. These issues may result in lower registration rates for the NDSS among First Nations people. For example, programs operating under Section 100 of the *National Health Act 1953* – such as Aboriginal Medical Services and the National Aboriginal Community Controlled Health Organisation – provide First Nations people access to free and subsidised products that people with insulin-treated diabetes need.

Remoteness area

In 2021, the age-standardised prevalence rate of diabetes was highest in *Remote and Very Remote* areas where people were 1.3 times as likely to be living with diabetes as those in *Major Cities*. The disparity in *Remote and Very Remote* areas was more pronounced among females than males (1.6 and 1.1 times as high, respectively) (Figure 4).

Socioeconomic area

In 2021, the age-standardised prevalence rate of diabetes was around 1.8 times as high among those living in the lowest socioeconomic areas as in the highest socioeconomic areas. The variation in prevalence rates between the lowest and highest socioeconomic areas was slightly higher among females than males (2.0 and 1.7 times as high, respectively) (Figure 4).

Figure 4: Prevalence of diabetes, by selected population groups and sex, 2021

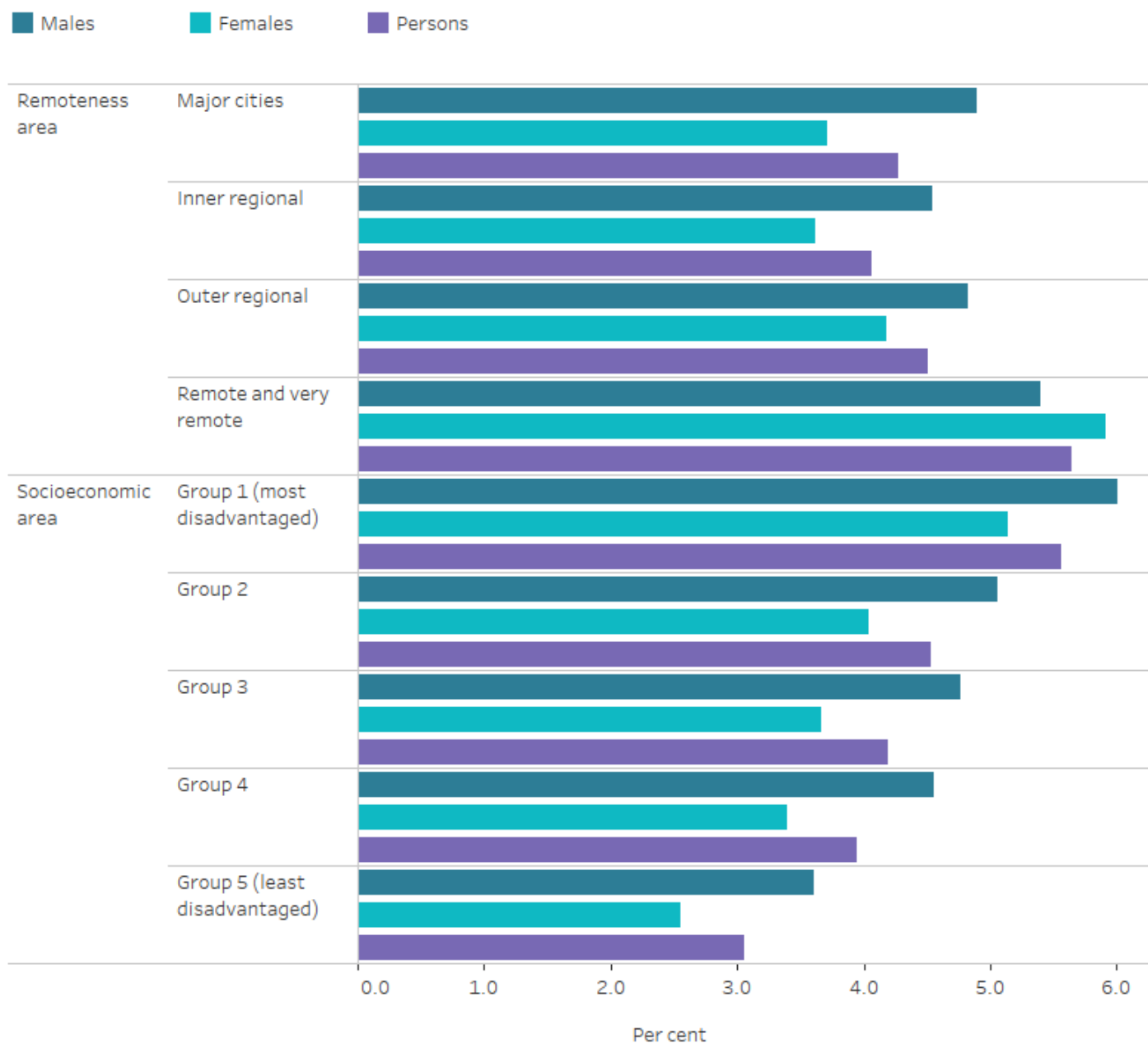


Chart: AIHW. Source: AIHW analysis of National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

<https://www.aihw.gov.au>

Notes

1. Age-standardised to the 2001 Australian Standard Population.
2. Remoteness is classified according to the Australian Statistical Geography Standard 2016 Remoteness Areas structure based on postcode of current residence.
3. Socioeconomic areas are classified according to population-based quintiles using the Index of Relative Socio-Economic Disadvantage (IRSD) based on Statistical Area Level 2 (SA2) current residence.
4. Includes persons with missing or unassigned information on age and/or sex.

How many people are newly diagnosed with diabetes in Australia?

Between 2000 and 2021, over 1.4 million people newly diagnosed with diabetes (incidence) were registered on the NDSS and APEG state-based registries, including type 1 diabetes, type 2 diabetes and other diabetes but excluding gestational diabetes. This is about 64,000 people each year – an average of about 175 diagnosed individuals per day. In 2021, 49,900 people were newly diagnosed with diabetes in Australia (137 per day), equating to 194 per 100,000 population.

Variation by age and sex

In 2021, the incidence of diabetes:

- increased with increasing age, peaking in the 65–69 age group for both males and females (532 and 392 per 100,000 population, respectively) (Figure 5)
- was 1.4 times as common among males as females overall, after controlling for age.

Figure 5: Incidence of diabetes, by age and sex, 2021

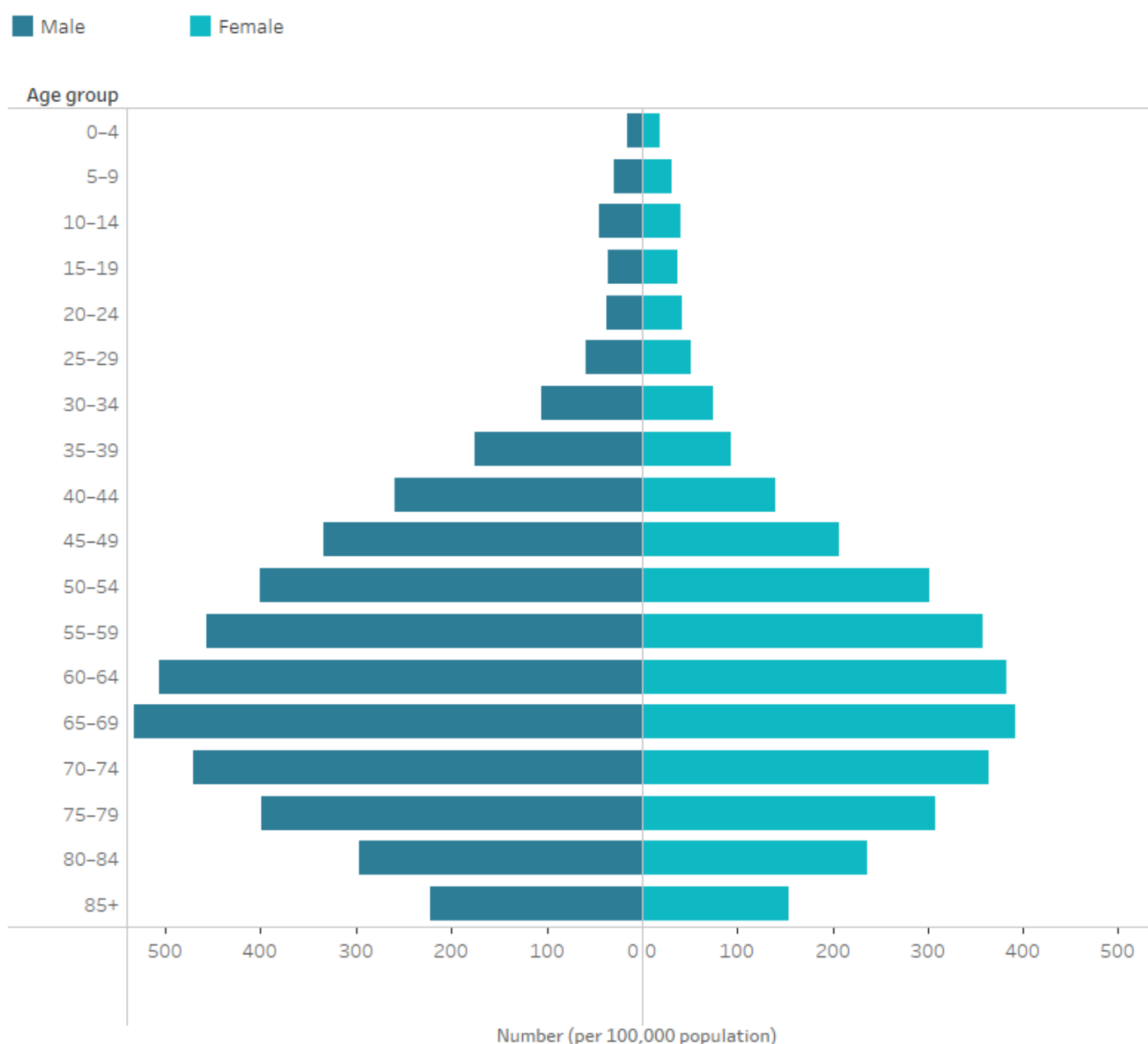


Chart: AIHW. Source: AIHW analysis of linked National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

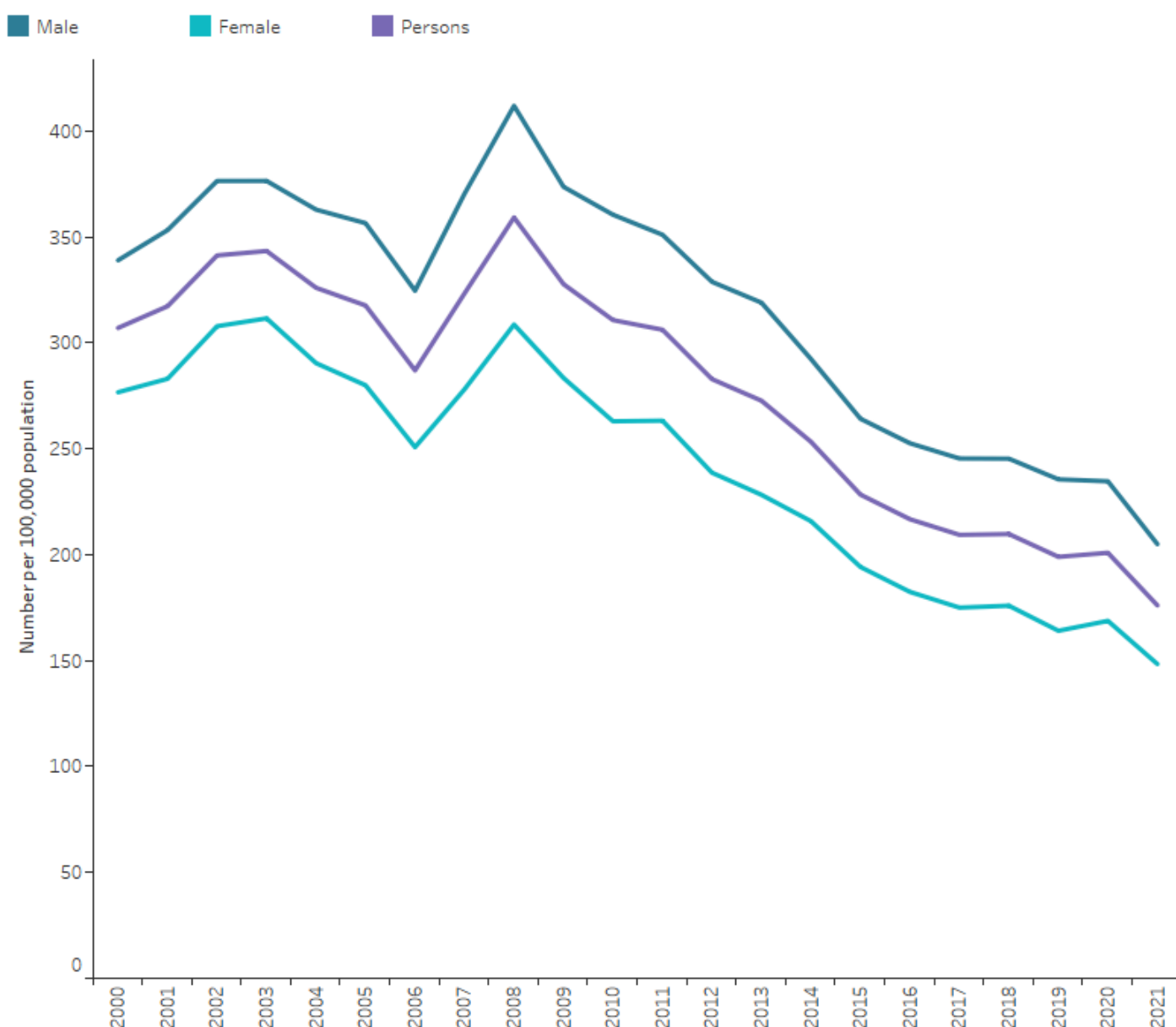
<https://www.aihw.gov.au>

Trends over time

There has been a steady decline in the age-standardised diabetes incidence rate in Australia (largely driven by type 2 diabetes) between 2000 and 2021, with an overall drop of 43% (Figure 6). Although evidence from the 2019 Global Burden of Disease study as reported in *The Lancet* (2020) shows an overall increase in diabetes incidence globally, the observed declining trend has also been reported in several other countries (Magliano et al. 2019; Magliano et al. 2021). The fall in incidence may be due to improved preventive measures such as screening, increased awareness and educational programs leading to behavioural changes and risk factor modification (Magliano et al. 2019; Shrapnel and Butcher 2020).

Note: Some caution should be used when interpreting these trends. The NDSS is estimated to capture 80–90% of all people with diagnosed diabetes in Australia (AIHW 2009). It is uncertain how many people with diagnosed diabetes are not registering with the NDSS and how many people are living with undiagnosed diabetes. First Nations people are also known to be under-represented on the NDSS.

Figure 6: Incidence of diabetes, by sex, 2000–2021



Note: Age-standardised to the 2001 Australian Standard population.

Chart: AIHW. Source: AIHW analysis of linked National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

<https://www.aihw.gov.au>

Variation between population groups

First Nations people

In 2021:

- there were around 2,200 new cases of diabetes among First Nations people – a rate of 256 per 100,000 population
- incidence rates increased with increasing age and peaked among those aged 65–69 (788 per 100,000 population)
- incidence rates were highest in the 65–69 age group in First Nations males but in the 60–64 age group among First Nations females (925 and 712 per 100,000 population, respectively) (Figure 7)
- incidence rates were 1.2 times as high among First Nations males compared with First Nations females, after controlling for age
- after adjusting for differences in the age structure of the populations, the incidence of diabetes was 2.0 times as high among First Nations males and 2.4 times as high among First Nations females compared with their non-Indigenous counterparts (Figure 8).

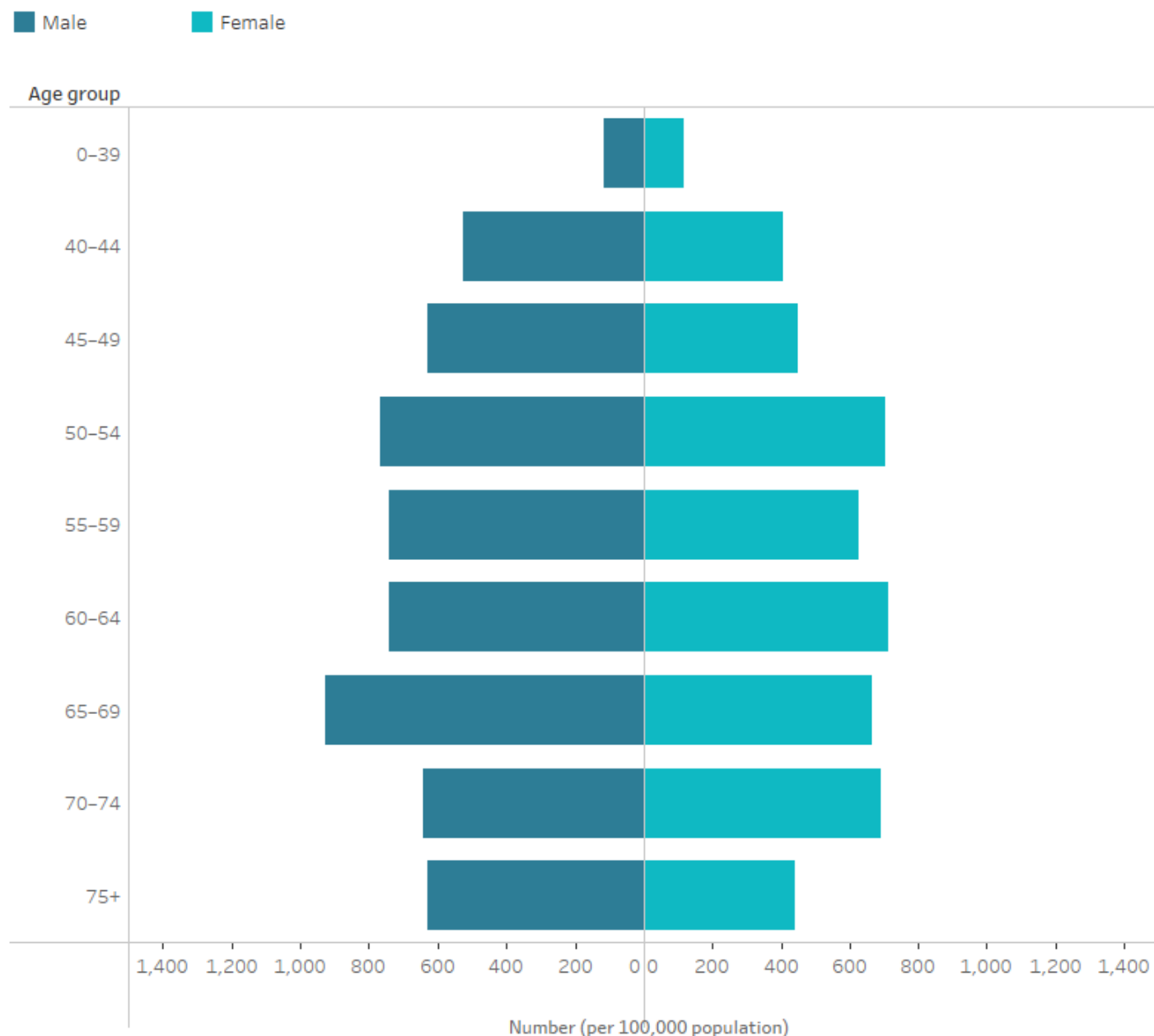
Figure 7: Incidence of diabetes for First Nations people, by age and sex, 2021

Chart: AIHW. Source: AIHW analysis of linked National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.

<https://www.aihw.gov.au>

Remoteness area

In 2021, age-standardised diabetes incidence rates were around 1.1 times as high in *Outer Regional areas* compared with *Major Cities* and *Inner Regional areas* and 1.2 times as high as *Remote and Very Remote areas* (Figure 8).

Socioeconomic area

In 2021, age-standardised diabetes incidence rates increased with increasing levels of socioeconomic disadvantage. People living in the lowest socioeconomic areas were 1.9 times as likely to be diagnosed with diabetes as those living in the highest socioeconomic areas (Figure 8).

Figure 8: Incidence of diabetes, by selected population group and sex, 2021

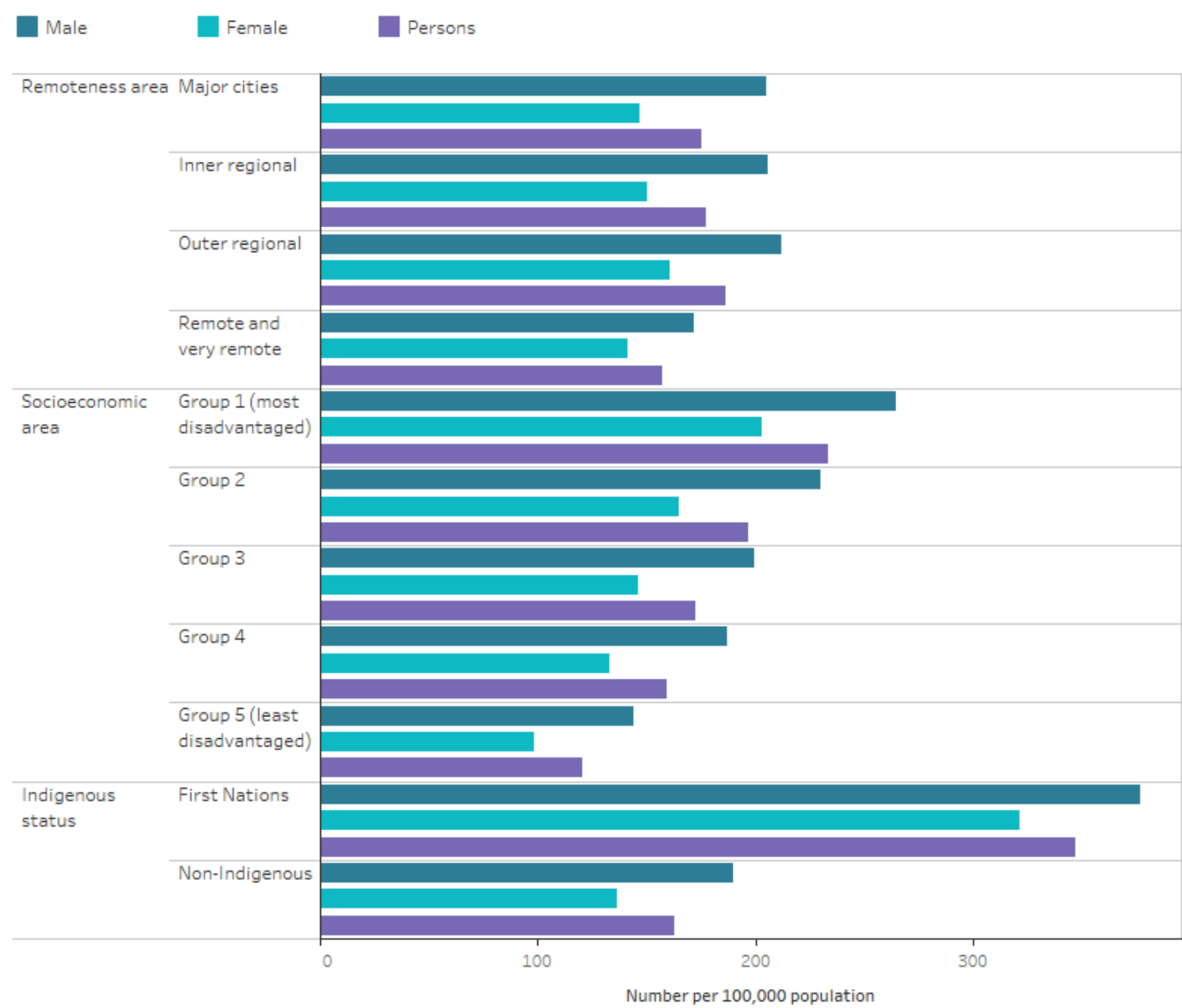


Chart: AIHW. Source: AIHW analysis of National Diabetes Services Scheme and Australasian Paediatric Endocrine Group state-based registers.
<https://www.aihw.gov.au>

- Notes
- 1. Age-standardised to the 2001 Australian Standard Population.
 - 2. Includes persons with missing or unassigned information on age and/or sex.
 - 3. Excludes persons whose Indigenous status, remoteness area and/or socioeconomic area was missing or unassigned.
 - 4. Remoteness is classified according to the Australian Statistical Geography Standard 2016 Remoteness Areas structure based on postcode of current residence.
 - 5. Rates may be influenced by the lower capture of First Nations people and those living in *Remote and Very Remote* areas or across states and territories with large remote communities on the National Diabetes Services Scheme.

References

ABS (Australian Bureau of Statistics) (2003) Microdata: *National Health Survey, 2001*

(<https://www.abs.gov.au/ausstats/abs@.nsf/ProductsbyReleaseDate/2EAF21C0E6710A54CA257107007819F8?OpenDocument>), AIHW analysis of detailed microdata.

ABS (2009) *Microdata: National Health Survey, 2007–08*

(<https://www.abs.gov.au/AUSSTATS/abs@.nsf/Lookup/4324.0.55.001Main+Features12007-08?OpenDocument>), AIHW analysis of detailed microdata, accessed 1 December 2021.

ABS (2013a) *Australian Health Survey: biomedical results for chronic diseases, 2011–12*

(<https://www.abs.gov.au/ausstats/abs@.nsf/Lookup/4364.0.55.005main+features12011-12>), ABS website, accessed 1 December 2021.

ABS (2013b) *Microdata: Australian Health Survey–National Health Survey, 2011–12*

(<https://www.abs.gov.au/ausstats/abs@.nsf/Lookup/4324.0.55.003main+features12011-12>), AIHW analysis of detailed microdata, accessed 1 December 2021.

ABS (2014) *Australian Aboriginal and Torres Strait Islander Health Survey: First results, Australia, 2012–13*

(<https://www.abs.gov.au/ausstats/abs@.nsf/mf/4727.0.55.001>), ABS website, accessed 4 March 2022.

ABS (2016) *Microdata: National Health Survey, 2014–15*

(<https://www.abs.gov.au/ausstats/abs@.nsf/Lookup/4324.0.55.001main+features12014-15>), AIHW analysis of detailed microdata, accessed 4 March 2022.

ABS (2019) *Microdata: National Health Survey, 2017–18* (<https://www.abs.gov.au/statistics/microdata-tablebuilder/available-microdata-tablebuilder/national-health-survey>), AIHW analysis of detailed microdata, accessed 4 March 2022.

ABS (2022) *National Health Survey* (<https://www.abs.gov.au/statistics/health/health-conditions-and-risks/national-health-survey/2022>), ABS website, accessed 2 October 2024.

AIHW (Australian Institute of Health and Welfare) (2009) *Diabetes prevalence in Australia: an assessment of national data sources*, AIHW, Australian Government, accessed 06 June 2022.

Dunstan D, Zimmet P, Welborn T, Sicree R, Armstrong T, Atkins R, Cameron A, Shaw J and Chadban S, on behalf of the AusDiab Steering Committee (2001) *AusDiab Report 2000: Diabetes & associated disorders in Australia–2000. The accelerating epidemic* (<https://www.baker.edu.au/impact/ausdiab/resources>), The Australian Diabetes, Obesity and Lifestyle Study (AusDiab), International Diabetes Institute, accessed 6 December 2021.

Magliano DJ, Islam RM, Barr EL, Gregg EW, Pavkov ME, Harding JL, Tabesh M, Koye DN and Shaw JE (2019), 'Trends in incidence of total or type 2 diabetes: systematic review' (<https://www.bmj.com/content/366/bmj.l5003>), *BMJ*, 366:l5003, doi:10.1136/bmj.l5003

Magliano DJ, Chen L, Islam R, Carstensen B, Gregg E, Pavkov M, Andes L, Balicer R, Baviera M, Boersma-van Dam E, Booth GL, Chan JCN, Chua YX, Fosse-Edorh S, Fuentes S, Gulseth HL, Gurevicius R, Ha KH, Hird TR, Jermendy G, Khalangot MD, Kim DJ, Kiss Z, Kravchenko VI, Leventer-Roberts M, Lin CY, Luk AOY, Mata-Cases M, Mauricio D, Nichols GA, Nielsen MM, Pang D, Paul SK, Pelletier C, Pildava S, Porath A, Read SH, Roncaglioni MC, Lopez-Doriga Ruiz P, Shestakova M, Vikulova O, Wang KL, Wild SH, Yekutieli N, Shaw JE (2021) 'Trends in the incidence of diagnosed diabetes: a multicountry analysis of aggregate data from 22 million diagnoses in high-income and middle-income settings' ([https://www.thelancet.com/journals/landia/article/PIIS2213-8587\(20\)30402-2/abstract](https://www.thelancet.com/journals/landia/article/PIIS2213-8587(20)30402-2/abstract)), *The Lancet Diabetes and Endocrinology*, 9(4):203–211, doi:10.1016/S2213-8587(20)30402-2.

Shrapnel WS and Butcher BE (2020) 'Sales of Sugar-Sweetened Beverages in Australia: A Trend Analysis from 1997 to 2018' (<https://www.mdpi.com/2072-6643/12/4/1016>), *Nutrients*, 12(4):1016, doi:10.3390/nu12041016.

The Lancet (2020)  [GBD cause and risk summaries: Diabetes mellitus–Level 3 cause](https://www.thelancet.com/pb-assets/Lancet/gbd/summaries/diseases/diabetes-mellitus.pdf) (<https://www.thelancet.com/pb-assets/Lancet/gbd/summaries/diseases/diabetes-mellitus.pdf>), The Lancet website, accessed 24 May 2023.

Last updated 26/11/2024 v56.0

© Australian Institute of Health and Welfare 2026

